



Chair Health Committee with H.E Fardowsa

CECM Health with H.E Fardowsa

Implementation organization: Fountain of Hope (FoH)

Okoa Mama na Mtoto Initiative (OMMI)

Implementation Period: January – October 2025

Location: Garissa County, Kenya

Comprehensive Year I Implementation Report

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Acknowledgment

Acknowledgment of ICRHK's support in Year 1

Fountain of Hope sincerely appreciates the unwavering support and guidance provided by ICRHK throughout the Year 1 implementation. We extend our heartfelt gratitude and say thank you for walking this journey with us.

Acknowledgment of County leadership support in Year 1 implementation

We extend our sincere appreciation to the County Health Leadership for their unwavering support and collaboration throughout the Year 1 implementation. Their commitment, strategic direction and continued engagement have been instrumental in driving progress and strengthening maternal and newborn health interventions across the county. We are grateful for their partnership and dedication to improving the well-being of our communities.

Acknowledgment of Community support in Year 1 implementation

We sincerely thank the community members whose support and participation made the successful implementation of Year I possible. Your commitment, insights and collaboration have strengthened our work and inspired every milestone achieved. We are grateful for your partnership and look forward to continuing this journey together.

Executive Summary

In its first year of implementation, the Fountain of Hope Organization (FoH), in partnership with the ICHRK through the Okoa Mama na Mtoto Initiative (OMMI) delivered a targeted and high-impact package of advocacy and community engagement activities across Garissa County aimed at reducing maternal, newborn and child mortality. These efforts brought together county leadership, policymakers, community influencers and local media to strengthen governance, raise public awareness and foster community ownership of maternal and newborn health priorities. Through coordinated dialogues, evidence-based reviews and targeted engagement the initiative successfully built momentum for sustained action and positioned local stakeholders at the center of driving long-term transformative change in MNCH outcomes.

Year One centered on strengthening partnerships, enhancing awareness and cultivating genuine community ownership in maternal and newborn health (MNH).

During this period, Fountain of Hope successfully implemented a series of strategic interventions including:

1. An inception meeting with County Health Leadership
2. County and Sub-County landscaping reviews to assess the MNCH burden using available data
3. A joint high-level executive meeting to align priorities and commitments
4. Advocacy engagements with MCAs and the County Health Leadership Team
5. Identification, onboarding and sensitization of MNCH champions
6. Advocacy dialogues with women religious leaders to amplify community influence
7. Collaboration with local radio stations to expand public awareness and reach

Other activities within the County where FOH participated and give highlights included

- Maternal and newborn monthly audits
- County MPDSR report writing
- County performance review meeting
- Capacity building of health care workers

By implementing these interventions, Fountain of Hope has established a solid foundation for transforming and improving maternal, newborn and child health services across the county.

Despite contextual challenges including cultural barriers and resource constraints the project delivered significant achievements in community engagement and policy influence. The progress, insights and lessons generated during this foundational year now serve as a strategic roadmap, positioning the initiative for even more impactful, coordinated and scalable interventions in Year Two.

Introduction

Garissa County, located in Kenya's North Eastern region, continues to record some of the highest maternal and newborn mortality rates nationwide. The county's vast terrain, highly mobile pastoralist communities and persistent infrastructural gaps have made it difficult for women and children to access consistent quality healthcare.

In response to these challenges, the Okoa Mama na Mtoto Initiative (OMMI) was established to accelerate progress toward Sustainable Development Goal 3 (Good Health and Well-being). This initiative aims to reduce preventable maternal and child deaths by strengthening advocacy, enhancing service delivery and reinforcing accountability mechanisms across the health system.

As a key implementing partner, FoH has focused on:

- **Advocating for improved maternal and newborn health outcomes** through evidence-based dialogue.
- **Strengthening community systems and leadership involvement** for local ownership.
- **Mobilizing faith and youth networks** to promote safe motherhood and child survival.

Throughout year 1 implementation, the organization worked closely with the County Department of Health, County executive, County Assembly, religious institutions and

youths to ensure that the OMMI initiative aligns with local priorities and effectively responds to the MNCH realities of Garissa communities.

Key Achievements from Year One

The first year of implementation was marked by a series of coordinated activities that strengthened partnerships, improved awareness and enhanced community participation in maternal and newborn health advocacy.

1. County Health Team Landscaping and Data Analysis

In April 2025, FoH collaborated with the County Health Management Team to conduct a detailed analysis of maternal and child health data. The exercise reviewed service utilization trends, facility readiness and health outcomes. It provided an evidence-based foundation for decision-making and planning.

The meeting brought together over 30 participants, including County and Sub-County management teams responsible for coordinating and managing maternal and child health services. The analysis revealed encouraging improvements, such as increased immunization and skilled delivery coverage but also highlighted persistent gaps in access and quality of care.

From the discussions, the health team agreed to enhance data feedback to facilities, raise public awareness through community structures and strengthen capacity building for health workers to sustain progress.

The team also developed key action points informed by the available data and responsibilities were distributed among the management members. The priority actions identified included strengthening the availability of blood and blood products, improving the efficiency of referral services and ensuring timely decision-making for effective patient management.

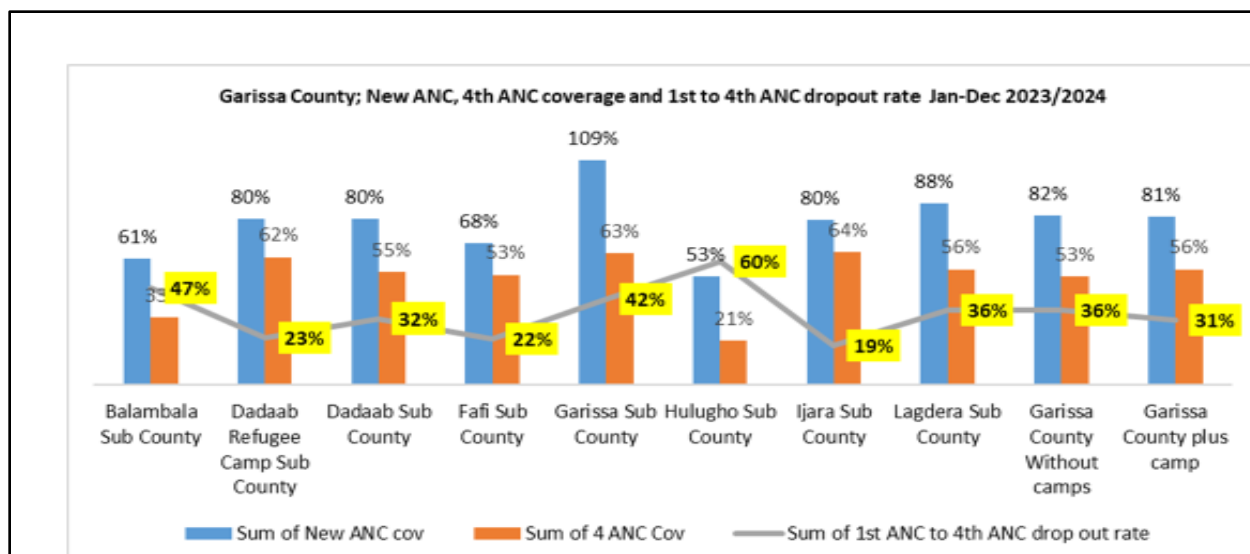


Figure 1: Comparison of 1st and 4th ANC coverage and drop out rate

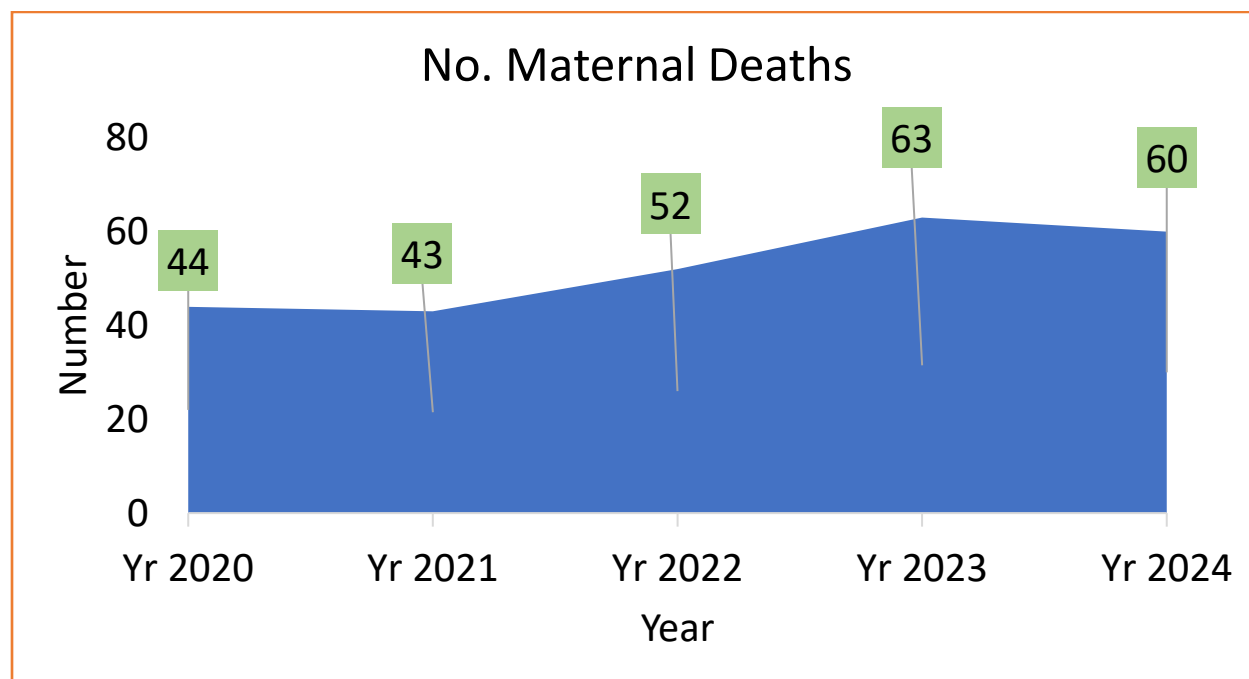


Figure 2: County Maternal mortality deaths trends (Numbers)

County Director of Health Quote during the meeting

"The persistently high maternal and child mortality rates in Garissa remain one of our most urgent challenges. We must come together and strengthen our responsive systems in Maternal and Child health services. Our priority is to close the existing gaps in referral, blood availability and timely decision-making so that no mother or child is lost to preventable causes."

2. Advocacy Engagement with Women Religious Leaders

Recognizing the influential role of religion in shaping community norms, FoH convened forty (40) women religious leaders in May 2025 for a consultative meeting on maternal and newborn health. The session aimed to align faith-based leadership with public health priorities and empower women of faith as advocates for safe motherhood.

During the engagement, leaders were sensitized on OMMI objectives and equipped with information on antenatal care, facility-based deliveries and newborn care. Many pledged to integrate maternal health messages into religious forums, sermons and women's gatherings.

This partnership between FoH and the women's faith network was an important breakthrough creating a channel through which health information could be delivered in culturally resonant and trusted spaces.

Quote from one of the women religious leaders

“Our religion Islam allows us to do child spacing and breastfeed our baby for 24 Months. This is meant to take care of the mother health and give time for the child to grow. I am telling our mothers they need to go to facilities for antenatal care so that any problem can be detected early by the doctors. We also need to come to hospital during labour. We will use our gathering to inform our congregation about what we learnt in this meeting.”



County RH Coordinator with the women religious leader's engagement

3. Advocacy Meeting with MCAs and County Health Leadership

In August 2025, FoH organized an advocacy meeting with Members of the County Assembly (MCAs), the CECM for Health, Chief Officer, and Director of Health. The purpose was to deliberate on MNCH financing, resource allocation and policy coordination.

The session revealed that although the health sector receives the largest portion of the county budget, over 70% of the funds are spent on salaries, leaving limited resources for service delivery. Participants agreed on the need for innovative financing models and stronger advocacy to prioritize maternal and newborn health programs.

A major issue discussed was the burden of cross-county referrals, particularly from neighboring Tana River County where deaths of referred mothers were often recorded as Garissa's, inflating the county's mortality statistics. The health leadership and MCAs committed to developing joint frameworks for accurate data reporting and to pushing for fairer distribution of health funds in subsequent budgets.

The Honorable Members underscored the importance of sensitizing and engaging the wider County Assembly to secure their full buy-in noting that this approach will expand opportunities to reach more mothers across all wards. They further committed to presenting the matter before the Assembly for deliberation and consideration during resource allocation.



Chair, County assembly Health Committee receiving project documents from H.E Madam Fardowsa



The Health leadership and County assembly health committee members

Quote from Chair health committee

“We appreciate the health leadership and Fountain of Hope in collaboration with OMMI for highlighting the burden of maternal and child deaths in this county. Indeed, this is an opportunity to bring some improvement so that we don’t lose more mothers. We as County assembly members especially I and my other health committee members commit to championing maternal health by advocating for adequate financial resources and ensuring that every part of our health system functions effectively. Our work is oversight and accountability to all department.”

4. Youth Champions Engagement on Maternal and Newborn Health

In June 2025, FoH trained fifteen youth champions as peer educators and advocates for maternal and newborn health. The training covered topics such as antenatal and postnatal care, the importance of skilled delivery and identifying danger signs during pregnancy.

The youth were encouraged to become active agents of change by:

- Using digital platforms like WhatsApp, TikTok, and Facebook to share health messages.

- Mobilizing their peers and families to support pregnant women in accessing care.
- Promoting girls' education and discouraging early marriages as long-term interventions.

The youth engagement also promoted male participation in maternal health encouraging young men to support their partners during pregnancy and childbirth.



Youth Champion sensitization

As a result, the youth champions emerged as visible agents of change that will amplify maternal health messages, breaking down stigma around family planning and encouraging greater trust in hospital deliveries.

5. Radio Talk Show on Maternal and Newborn Health Advocacy

In July 2025, FoH partnered with Star FM, one of the most popular local radio stations in Garissa and that has over 70% coverage to broadcast a live talk show focusing on maternal and newborn health. The program, aired in both Somali and Swahili, reached thousands of listeners across the sub-counties in Garissa County.



Radio talk show and calling in session

Listeners called in to discuss topics such as antenatal visits, skilled delivery, family planning and male participation in maternal health. They also called to ask about the quality of services provided. The show proved to be an effective advocacy tool for the population to understand more on why our mothers are dying and the need to visit hospitals during pregnancy.

Some of the callers' questions included;

- What happens during a pregnancy visit and why is it important?
- Why should my wife deliver in a health facility instead of home?
- If my wife is bleeding during pregnancy that is normal and what can I do?
- Is giving birth at hospital free?

Through the many radio sessions conducted the messages have reached an estimated audience of more than 300,000 people

The success of this broadcast confirmed that radio remains one of the most powerful platforms for influencing public perception particularly in pastoralist settings where in-person outreach can be limited.

6. Advocacy Meeting with County Executive and Health Leadership Team (October 2025)

As part of its ongoing advocacy, FoH held a high-level engagement meeting with the County Executive and senior health leadership on 29th October 2025. The meeting, chaired by the Deputy County Secretary, aimed to mobilize executive-level commitment to improving maternal and newborn health outcomes.

The meeting brought together high-level representation from the executive and the health leadership including:

- Deputy County Secretary
- Representation from CECMs
- Chief Officer, Health
- Director of Health
- Head: Family Health
- Head: Curative and referral services
- Representation from Health Committee at the Assembly
- Representation from Budget Committee Members
- Head: Policy and Planning, M&E
- County health management team
- Fountain of Hope OMMI Advocacy Team

The County Health Director presented data on MNCH performance, highlighting key challenges such as human resource shortages, infrastructure gaps and limited financing. The CECM representative emphasized that this forum was not just for discussion but a platform for action, urging stakeholders to align Garissa's strategies with the "*Every Newborn Action Plan* and *Ending Preventable Maternal Mortality*" framework.



Key advocacy priorities included:

- Increasing domestic financing for RMNCAH.
- Expanding Social Health Authority (SHA) coverage for expectant mothers.
- Strengthening community health strategies through CHPs.
- Enhancing data systems for real-time monitoring of maternal and newborn indicators.
- Adopting MNCH scorecards to track progress and accountability
- Sharing quarterly reports with the executive

The meeting concluded with firm commitments to conduct quarterly reviews, upgrade rural health facilities, adequate supply of essential commodities on quarterly basis and ensure round the clock access to skilled birth attendance. In his closing remarks, the Deputy County Secretary reaffirmed Garissa's dedication to driving inclusive and transformative change in maternal and newborn health declaring that no mother or child should be left behind.

Challenges and Lessons Learned

Despite the notable progress achieved several challenges persisted during the implementation

Among the most significant were:

- **Deep-rooted cultural norms and Beliefs** that continue to discourage women from delivering at health facilities. There is strong preference for home deliveries and reliance on traditional birth attendants.
- **Political Prioritization Challenges**-Competing County priorities can overshadow maternal health advocacy
- **Insecurity in certain areas** that restricts movement of mothers and health care workers
- **Weak data use for advocacy**- Limited capacity by the health managers to translate data into compelling advocacy messages
- **Limited male involvement** in decision-making related to maternal health, leaving many women unsupported.
- **Inadequate health financing**, with most resources directed toward recurrent expenditure rather than service delivery.

From these challenges emerged several critical lessons:

- **Inclusive engagement works.** Building partnerships across government, faith, and community sectors creates stronger accountability and ownership.
- **Faith-based advocacy is effective.** Aligning health promotion with cultural and religious values increases community acceptance and participation.
- **Youth leadership matters.** Young people, when empowered can drive awareness and behavioral change through innovative communication channels.
- **Information access saves lives.** Media platforms, particularly radio, are vital for reaching dispersed populations and normalizing discussions around maternal health.

Opportunities for Consideration in Year Two

As FoH transitions into Year Two of implementation, several opportunities present themselves for scaling and sustainability.

- **Deepening collaboration with policymakers:** Strengthening engagement with the County Assembly and Health Department will be key to advocating for improved MNCH funding and ensuring that resources are efficiently utilized.
- **Expanding faith-based and youth advocacy:** Building on the networks established in Year One, FoH will train additional faith leaders and youth champions across sub-counties to broaden the advocacy base.
- **Improving data systems and accountability:** Supporting the county in establishing robust data verification mechanisms will enhance transparency and evidence-based decision-making.
- **Sustaining community engagement:** Quarterly radio programs and localized community dialogues will continue to amplify key health messages.
- **Integrating MNH with related sectors:** Collaborating across nutrition, water and sanitation, and gender programs will ensure a holistic approach to maternal and child survival.

These opportunities position FoH and its partners to consolidate gains made in Year One and to make measurable strides toward reducing maternal and newborn mortality in Garissa County.

Conclusion

The first year of the Okoa Mama na Mtoto Initiative has demonstrated the transformative impact of coordinated advocacy, community mobilization and strategic partnership in addressing maternal and newborn health challenges.

Fountain of Hope has successfully built bridges between health institutions, policymakers, religious leader, and young people bringing collective focus to the urgent goal of saving

mothers and newborns. The initiative has not only created awareness but also empowered communities to take ownership of their health outcomes.

As the program enters its second year, FoH remains committed to sustaining this momentum. The organization envisions a future where every woman in Garissa County has access to skilled, compassionate care and every child is born into a community that values and protects life. Through collaboration, data-driven advocacy and inclusive engagement this vision is steadily becoming a reality.